JOHNS HOPKINS
BLOOMBERG SCHOOL
of PUBLIC HEALTH

CAHMI
The Child & Adolescent Health
Measurement Initiative

Department of Population, Family and Reproductive Health
Christina Bethell, PhD, MBA, MPH, Professor, The Child & Adolescent Health Measurement Initiative

June 16, 2025

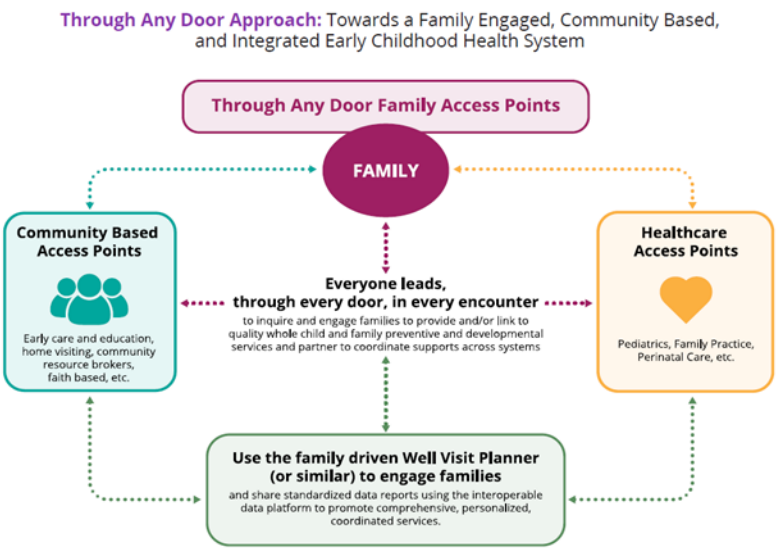
To: Whom It May Concern at: https://www.regulations.gov/document/CMS-2025-0050-0031
From: Christina Bethell, PhD, MBA, MPH Center for Early and Lifelong Flourishing (CELF), Child and Adolescent Health Measurement Initiative (CAHMI), Johns Hopkins University; cbethell@jhu.edu

Thank you for this opportunity to provide input about the necessity and possibilities to leverage the nation's existing and ever evolving digital health ecosystem to improve the health and wellbeing of adults, children and the population. I am writing with specific emphasis on the following:

1. ***O*pportunities** to advance a digital health ecosystem that meaningfully and **productively engages patients/families (using personal data they own and choose to share) and communities (using population data they can own and analyze)** in ways that implement behavioral economics principles to achieve much needed leaps in health outcomes
2. **Possibilities** to establish a digital health ecosystem that enables individual and population-based data sharing both **within health care settings (vertical integration) and across the many community-based services and public health programs (horizontal integration)** that are essential to address the many psychosocial, behavioral, environmental and practical daily support needs that drive poor outcomes and diminished well-being.

Given the word limit for this comment, I am attaching a document with links to brief videos and overview documents that further illustrate an example of an innovation that addresses the two priorities. I also welcome questions and to share more about the 30- year journey reflected in what is offered here.

In this brief comment, I illustrate possibilities to achieve the goals listed above by briefly sharing about the evidence-based ***Cycle of Engagement (COE)*** patient-facing, personalized digital health *Well Visit Planner* approach anchored to the nation's Bright Futures Guidelines and the related national system's transformation ***Engagement In Action (EnAct!) Framework*** that was created through Congressional language in 2017 to advance a national model for creating statewide integrated health systems that promote the healthy development of children. The COE and EnAct! framework enable a ***"Through Any Door"*** approach and centers the use of digital health technologies that enable personalized, whole person care and details the many leadership, cultural, technical and policy issues involved in advancing a health care system aiming to foster whole health and wellbeing. Doing so innately requires partnerships across systems, reductions in data collection/sharing burden and a focus on the whole person/family/child/youth where they are engaged to identify and partner to address their personal goals, needs, priorities and context. While created for children and families, the COE approach is relevant for all ages and health issues (it was recognized by ONC in 2012 as such and

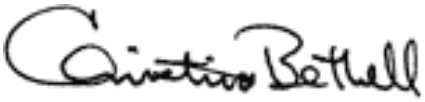


in the recent NASEM Launching Lifelong Health study). While this RFI focuses on Medicare, we respectfully share about this work as an example since it is relevant for all ages and health issues. Secondly, we share this also to urge a focus on children in your work since primary prevention in childhood using national Bright Futures Guidelines is a high leverage opportunity for improving the health of the adults and others with Medicare insurance coverage in the years to come. See the attached document with links to videos and short documents for more information. To summarize, The Cycle of Engagement Well Visit Planner approach (COE/WVP) is specifically designed to:

1. Engage patients using a **brief, time saving pre-visit** (or waiting/room/visit room) digital health tool produces a guidelines-based, whole person profile of health, risks, needs, goals and priorities.
2. Use of an **autogenerated Clinical Summary** that is digitally shared with providers and families providing an at-a-glance view (patients also get this) with resource links that enables "**Personalized Connected Encounters"** that implement best practice engagement, education, shared decision making and behavior change strategies with time to focus on resource sharing and linkages.
3. Provides a **validated post-visit** quality of care tool the periodically generates actionable quality reports to improve care and respond to value based care incentive programs.
4. Provides **population-based data** for identifying needs and planning systems
5. Features a **data sharing platform** patients can agree providers use to share their Clinical Summary and care plan across their care team and with others
6. Has been/can be **fully integrated with existing EHR systems but is not scaled** due to lack of incentives and focus on patient engagement, whole person care focused on prevention (and in our case, children).

With the United States ranking among the lowest in health and wellbeing for adults and children (roots of adult health) it is urgent to leverage strategies that build trust and relationships with patients and families in ways that effectively activate personal, family and community agency and will to be well and to gain skills and connect with available resources and supports to prevent illness and promote wellbeing, even amid illness or risks. Putting power into the hands of patients to self-assess, pick priorities, identify their needs and share that information across their care team in a way that ensures it is based on best practice guidelines and will be honored and responded to is important and possible. Patient facing digital health tools are important but are not integrated into existing EHRs, especially for prevention and children. The Cycle of Engagement approach briefly summarized here aligns with expert recommendations on ways to advance a more effective digital ecosystem. As noted by Jenssen, et al. "A more effective EHR would serve as an electronic hub. Existing EHR infrastructures could provide the foundation upon which new technologies and approaches branch and extend, enabling more rapid and customizable innovation to better meet shifting stakeholder and end-user needs. Among many areas for improvement, key goals of innovation could include technology that relieves ambulatory primary care clinician documentation burden, identifies needs, and supports improved care coordination and outcomes, focused on the following key areas: identification of child and family care needs, decision support, documentation, care coordination, and family communication."

Sincerely,



Christina Bethell, PhD, MBA, MPH
Professor, Bloomberg School of Public Health and School of Medicine
Johns Hopkins University
Director, Child and Adolescent Health Measurement Initiative
National Data Resource Center for Child and Adolescent Health
615 N. Wolfe Street, Room E4152, Baltimore, Maryland 21205